

What About Me? Ethical Blind Spots in Prenatal Screening: A Sibling's Perspective

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Prenatal screening provides parents with valuable information about potential health conditions and disabilities in their unborn child, offering them greater control over shaping their family's future. While parents seek prenatal screening to gain insight and prepare for family planning, the test results alone cannot constitute a fully informed decision. In addition to this medical information, parents must consider the broader implications it may have on their family dynamics and incorporate these considerations into their planning process.

In her book *Choosing Between Possible Lives: Law and Ethics of Prenatal and Preimplantation Genetic Diagnosis*, disability scholar Rosamund Scott argues that the decision to continue or terminate a pregnancy given a positive test result should consider "the interests of the future child born," particularly when the child is expected to have a "severe disability" (Scott 63). If the child's quality of life is anticipated to be significantly low due to a disability, it might be reasonable for parents to consider termination (Scott 63). Scott emphasizes that moral reflection on this issue should take into account not only the interests of the potential child but also the prospective parents and the relationship between them (Scott 11). She further recognizes that many decisions are actually driven by "the interests of prospective parents" (Scott 68). For instance, in cases involving conditions like Down syndrome, which are less directly related to severe health complications, the decision to terminate may primarily reflect the parents' interests over the child's interest. Regardless of whose interests shape the decision, Scott argues that "decisions should be respected, and [parents'] autonomy supported" (Scott 68). She explains that the concept of parental autonomy extends beyond the decision of whether to have a child to include which child they choose to have. In this sense, their decision to continue or terminate a pregnancy should still be respected, just as parents have the freedom to decide whether or not they want to have children.

Tom Shakespeare, another prominent disability scholar, also advocates for parents' autonomy in his book *Disability Rights and Wrongs*. He states that "the principle of reproductive autonomy suggests that prenatal diagnosis and termination should be available" (Shakespeare 129). According to him, completely banning the prenatal screening test limits parents' autonomy, so he argues that although it might be ethically concerning, the option should still be provided. Moreover, because parents are the ones who "directly experience the consequences," (Shakespeare 132) he argues that they have the right to make an informed decision. Therefore, if parents are not ready for the child, "it is their right to forgo this future" (Shakespeare 128). While his argument centers on the experience and perception of parents, he also recognizes the perspectives of the potential child. Although Scott mentions the quality of life of the prospective child, Shakespeare answers the question on whether harm is done to the child who is born with disabilities. He claims that a child is "not harmed by being brought into existence, where the only

other alternative was not to exist at all” (Shakespeare 128). He suggests that it is intuitively wrong to claim non-existence is preferable to life with a disability when “life with disability can be good” (Shakespeare 128).

Both scholars’ claims include approval of prenatal screening tests as it facilitates parental autonomy. While their emphasis is slightly different, both scholars justify the decision to undergo prenatal screening and potentially terminate a pregnancy based on the interests of the parents and the potential child. I agree with their claim of availability of the screening test, as banning such testing would limit the freedom of parents to make informed decisions. However, I want to highlight the gap in their claim regarding the prenatal decision beyond the justification of prenatal screening which only centers on the interest of the parent and potential child with disabilities. Siblings, who will also be profoundly impacted by the outcome of these decisions, are left out of this ethical conversation. Here, Shakespeare’s claim that decisions should be made by those who will directly experience the consequences (Shakespeare 132) seems contradictory, as siblings—who will inevitably be involved in the life and care of the child with disabilities—are excluded from the decision-making process.

My paper aims to address this oversight by examining prenatal decision ethics through the lens of siblings’ experiences for a complete ethical conversation. First though, I want to make it clear that the purpose of this paper is not to judge the ethics of prenatal screening itself. I am focusing on the information provided by the screening test that indicates a disability, and discussing what considerations or awareness parents should have when making an ethical and responsible choice additional to the test result. Additionally, my paper assumes that prenatal screening tests are fairly accurate. Today, diagnostic tests can determine with greater than 99.9 percent accuracy whether or not a developing baby has a chromosomal difference (Genetic Alliance). Based on these statistics, my paper assumes accurate test results and thus does not address the ethics surrounding false positives or false negatives.

Before delving further into the ethical decision making around the sibling’s perspective, I would like to address the ethics of informed decision-making and define what constitutes an “informed decision.” According to Ali Abbas, in his book *Ethical Decision Quality: Building an Ethical Decision Culture*, the ethical decision-making process involves awareness. Abbas’s ethical guideline encourages us to ask whether we were aware of the potential consequences, whether we tolerated them, and whether we could have predicted any implications of our actions (Abbas 97). It is important to note that Abbas’s framework suggests that decisions are inherently based on past experiences. Any decision, by nature, involves predicting the future based on information from the past (Abbas 98). Here, prenatal screening plays a crucial role in narrowing down which past data to focus on. However, the standard for how “informed” a decision must be in order to be considered ethical is vague. It’s because outcomes themselves cannot be the standard of informed decision. Not only is the standard on positive outcomes vague, but also outcomes can still be considered negative even with careful consideration and examination of the past. Outcomes are never guaranteed or certain because the future doesn’t yet exist when we are making decisions.

Therefore, uncertainty persists even with sufficient data. While information is crucial to an informed decision, it cannot be strictly defined by the quantity of information alone.

In the case of prenatal decisions, the information I refer to includes the experiences of the child with disabilities and those around them, as well as research or statistics on disabilities that the fetus has tested positive for. Here, I argue that parents should consider more than just this information. Decisions are not just based on information, data, but also bias, which speeds up the decision making process (Abbas 6). Bias also arises from past experiences but is uniquely shaped by individuals' perspectives. Usually, bias is perceived as a negative factor in decision making. In the case of prenatal decisions, a parent's perception of disability inevitably shapes their choices, raising the question: can a decision still be considered ethical when it is influenced by bias? I argue that bias is rooted in lived experience and is, to some extent, unavoidable. Parents draw on their past experiences, personal knowledge, and understanding of their family's needs to make what they believe is the best decision for their child. In this context, Immanuel Kant's philosophy offers valuable insight. As he states in *Groundwork for the Metaphysics of Morals*, the moral worth of an action is determined by the principle or intention from which it arises, rather than by its consequences (Kant 42). We don't have full control of the outcome, but we do over the intention. If the intention was "pure" (Kant 42), we can consider the action to be just. Therefore, if a parent's bias arises from genuine concern and a desire to do what is best for their family, it should not automatically disqualify the decision from being ethical. In other words, bias is a natural outcome of trying to act responsibly. When past experience and data inform decision-making, it becomes difficult to draw a clear line between "bias" and "informed judgment." Therefore, I would like to define an "informed decision" as one that combines past experience with an individual's values and good intention to make the best decision. Informed decisions are inevitably influenced by personal biases, and this bias does not change the fact that the decision is still informed. An informed decision is the best stance we can take before the outcome turns out, thus can be considered as a standard of ethical decision.

Now, I will examine how siblings can be meaningfully incorporated into the informed decision-making process or whether it is even possible to include them meaningfully, given that the future is too uncertain and unpredictable. As we explored in the previous paragraph, the decision-making process involves considering existing data or experience, and that is the only thing we can rely on. This limitation arises from the nature of our one-directional timeline—from past to future. Since we cannot compare outcomes that do not yet exist, we must make the best prediction based on the past. Evaluating the ethics of two possible decisions based on the past data makes the ethics of the decision making process tangible. By examining the past cases of the potential outcome, we can get a good sense of the ethics implied in the choice. In the case of prenatal screening, there are two choices: continuation of the pregnancy or termination. By examining these existing cases, we can make an informed decision, and thus make an ethical decision. As I defined earlier, an informed decision involves good intention. Therefore, parents should make a decision in a direction that minimizes the suffering of the child, as another option—maximizing suffering—is intuitively unethical. With this in mind, the next part of the ethical discussion focuses on the hardship experienced by siblings when parents decide to continue a

pregnancy with a child who has disabilities, and whether it is ethical for parents to let this happen. To explore this, I build upon broader ethical debates about the "harm" experienced by children born with disabilities themselves when parents choose to give birth despite the positive test result from the screening test.

If a person with a disability is born, can they be considered harmed? According to Ali Abbas, harm is defined as "manipulation of others... making their life hard" (Abbas 87). This definition suggests a comparison between two states, where one is made worse off through intentional actions. While this concept can generally be used to determine whether harm has occurred, it seems somewhat ironic in the context of prenatal decisions regarding a child with disabilities. Abbas's claim can be expanded through the philosophical perspective on existence offered by David Heyd in *Genethics: Moral Issues in the Creation of People*. Heyd argues that "we cannot meaningfully compare existence and non-existence at the start of life" because non-existence lacks inherent value, making it difficult to assess its worth in relation to life (Heyd 37). In short, we cannot compare something with nothing. Since we cannot compare the postnatal experience of the child to the state of non-existence, the condition of harm outlined by Abbas is not met. By combining Heyd's logic with Abbas's definition of harm, I argue that no one can judge if harm was done to a child born with disabilities. This leads me to the same conclusion as Shakespeare—that no harm is done to the child with disabilities just by virtue of their existence (Shakespeare 129). Given that we cannot conclude harm from past data, our best prediction suggests that no harm will be done to a child born with disabilities. As such, there is nothing inherently wrong with parents' decision to continue a pregnancy when the fetus is diagnosed with a disability.

However, the question remains: If parents know their child will have a disability but still choose to give birth, does this reflect bad intentions—giving suffering to a child? My answer is no. This question often stems from negative perceptions of disability and the belief that disability equals suffering only (Shakespeare 98). I claim that suffering is part of any life and thus focusing on suffering itself doesn't do much in ethical conversation. Instead, ethical conversation depends on parents' perception of positive aspects of life that makes the life worth living. If a parent's perspective, values, and biases lead them to believe that the life of a child with disabilities is worth living, their intentions are not inherently bad or unethical. In fact, the choice can be seen as rooted in a belief in the intrinsic value of life, regardless of disability. From this standpoint, parents are making an ethical decision that reflects their desire to honor the child's life and potential, despite the challenges that disability might bring. Because giving birth to a child with disabilities cannot be viewed as parents' desire to maximize the suffering of the child, it doesn't disqualify them from making an ethical decision. Ultimately, the ethics of their decision lies in the sincerity of their intentions and the thoughtful consideration of the child's future, not in any preconceived notion of what constitutes a good life (Scott 68).

A similar logic of Heyd can be applied to the siblings of children with disabilities. Since Heyd's point centers on the inability to compare something that doesn't yet exist or is uncertain, we can extend his argument to the incomparability of unpredictable outcomes between terminating a pregnancy with a disabled child versus continuing the pregnancy. When siblings experience

hardship due to growing up with a child with disabilities, we cannot meaningfully compare this level of suffering with the potential outcome of what would have happened if the parents had made a different choice. We cannot experience the outcomes of both decisions—only one. As Heyd mentioned, it's not meaningful to compare an outcome that is certain or already experienced with another outcome that would have occurred if the parents made a different decision. Therefore, despite the hardships that siblings may experience, we cannot draw a meaningful comparison between the actual outcome and the potential outcome. Consequently, we can conclude that no harm is done to the siblings of a child with disabilities by giving birth to that child. By examining past cases and drawing upon available references, we can see that parents can ethically choose to continue the pregnancy for a fetus with potential disabilities, and the siblings will not be harmed by this decision.

However, the alternative outcome—terminating the pregnancy to prevent the birth of a child with a disability—might appear to be a more reasonable choice for the siblings. In this case, the comparison is between the actual experiences of children who have siblings with disabilities and those of children who have siblings without disabilities. Yet, I would argue that such comparisons are also meaningless. Everyone has different experiences, and even when the environment or circumstances seem similar, perceptions can differ drastically. To assess ethics and the “harm” to an individual, we must focus on personal experiences rather than comparing one person's situation to another's. Comparing the experiences of different individuals is meaningless, at least in a conversation that values the ethics of the action. It's because comparisons often fall short, as our assumptions are shaped by limited understanding.

To further illustrate this point, I would like to introduce the “disability paradox,” a concept that emphasizes how the quality of life for individuals with disabilities is often perceived differently from their actual subjective experiences (Schalock 1996). According to research, individuals with disabilities often report higher levels of life satisfaction than what is predicted by those without lived experience—and in some cases, even higher than the reported satisfaction of individuals without disabilities (Schalock 1996). This paradox occurs because, despite the challenges they face, many people with disabilities find meaning, joy, and purpose in their lives. Impairment can be hard to navigate, but life with impairment can be good (Shakespeare 121). Furthermore, “[i]t would be untrue to claim that...people without impairments do not suffer” (Shakespeare 131). As research shows, the perception of suffering or hardship is frequently amplified by societal attitudes and misconceptions about disability.

Additionally, I would like to highlight the duality of life which involves both suffering and growth. The complexity further makes it harder for comparison of experiences. While siblings face challenges, they can often grow through these experiences in unexpected ways. This complexity is highlighted in Jennifer Chase's article *The Sibling's Perspective: Experiences of Having a Sibling with a Learning Disability and Behavior Described as Challenging*. Despite the difficulties and responsibilities, many siblings report that their relationship with their sibling is “positive, loving, and strong” (Chase). Some siblings even express personal growth, with one stating, “I have become more independent or was more independent and therefore I was able to

cope with it” (Chase). Others reflect on how the experience strengthened their relationships, with one sibling noting, “I’d say it’s made us pretty close...[M]y mum and I...support each other” (Chase). These positive outcomes are frequently overlooked when we focus solely on the difficulties, ignoring the potential for personal growth and meaningful relationships. The outcomes of significant decisions are rarely binary—life is not simply good or bad. Siblings of children with disabilities do not always suffer, and the assumption that individuals without siblings with disabilities live without suffering is also false. Therefore, due to the incomparability of experience and the complexity of the outcomes, it is difficult to judge the life of a sibling of a child with disabilities as inherently bad. Since we can’t make meaningful comparisons between actual experiences due to their complexities, we can make our best prediction that the outcomes of future decisions will also be incomparable. In other words, we cannot identify any indication of harm—since harm requires comparisons—, and this choice can be ethically justified. Consequently, parents are not in a position to be blamed if a sibling of a child with disabilities experiences hardships.

Despite the duality of life, the negative associations with disability tend to be stronger (Shakespeare 87). However, as I argued earlier, these biases—stemming from negative perceptions of disability—cannot be fully blamed, as they are closely tied to the good intentions of parents trying to minimize their child’s suffering. It is true that parents may make decisions based on bias, subconscious perceptions, and countless comparisons, yet their decision can still be ethical. In this sense, their intention meets the condition of ‘pure’ intention in ethical decision-making. However, this does not reflect the full truth. Therefore, I argue that in the conversation about harm within ethical decision-making, we should not solely rely on bias—the seemingly ‘good’ or ‘bad’ outcomes of past decisions. Not only because of the duality of the outcomes, but also due to our inherently biased perceptions, we cannot accurately judge the goodness or badness of the outcomes. Instead, we can only assess the ethics of the decision at the moment it is made.

I want to highlight how Heyd’s explanation of the incomparability of existence versus non-existence, which I introduced earlier, can help separate the judgment of the outcome from the decision itself and allow for a more objective evaluation of the ethics. Since we are biased when perceiving others’ experiences, rather than comparing the experiences of different individuals, we must focus on each individual’s experience separately. By applying Heyd’s logic of existence versus non-existence to evaluate the harm done to each individual, I argue that bias will not influence our ethical conclusion. If we apply Heyd’s logic to each individual’s experience, regardless of any hardship the child may face, we arrive at the same conclusion: we cannot compare the value of existence and non-existence, and thus, no harm is done to the child. As long as the decision is informed, parents are inevitably making an ethical decision.

So far, I have demonstrated that no harm is done to siblings and highlighted the positive experiences they may have. However, my emphasis on this point is to highlight the neutral nature of siblings’ experiences with disabilities by balancing the positive aspects with the hardships they face, as the perception of disability is “not neutral” to begin with (Shakespeare).

While the word "disability" often provokes negative associations, I have shown that this assumption is not universally true. Therefore, the experiences of siblings are truly neutral, as the complexity of their experiences makes meaningful comparison difficult. With this in mind, I want to make a neutral claim. Regardless of the decision—whether to continue or terminate the pregnancy—some form of suffering may arise, whether the child has a sibling with disabilities or not. The inability to compare these outcomes still applies here. Any suffering that results from existence cannot be blamed on parents, because there is no meaningful comparison between the actual outcome and the potential of what could have been. According to this simulation driven from the past data, I argue that both possible prenatal decisions—whether to continue or terminate a pregnancy involving a child with disabilities—are ethically acceptable in siblings perspectives, and neither choice is ethically favored over the other. Thus, the decision can be considered truly neutral and ethical as long as it involves thoughtful consideration.

Up to this point, I have used Heyd's logic of existence versus non-existence as a central framework for my argument on the ethics of prenatal decisions from the perspective of siblings. However, regarding the application of this logic, critics may argue that the ethical weight of these decisions changes depending on the timing of the birth, suggesting that a different logic should apply to each scenario. For instance, when parents are considering terminating a pregnancy for a child with disabilities while they already have a child without disabilities, some might argue that—since the child without disabilities has already been brought into existence—the situation is no longer one of existence vs. non-existence, but rather existence vs. existence. Therefore, a different justification for non-harm may be required. However, the logic of existence and non-existence remains applicable. While the child without a disability has already been brought into the world, their future alongside a sibling with a disability remains uncertain and cannot be ethically compared to an alternative scenario that never occurred. Another concern may arise regarding the termination of a child with disabilities as it may be seen as a form of discrimination by valuing the life of a child without disabilities over the potential child with disabilities. However, I would argue that such a decision is not inherently discriminatory. Rather, it can be the result of a complex, reflective evaluation of family dynamics, including the needs and potential suffering of the existing child.

The relationship between siblings is dynamic and responsive. Heyd notes that "interdependence is even more conspicuous" in family planning decisions, where the well-being of one member significantly affects others (Heyd 94). Thus, if parents believe their current child would bear too much responsibility or emotional burden, their decision to terminate might reflect an assumption that the sibling relationship would be unmanageable—thus not beneficial for either the sibling without a disability or the potential child with a disability. On the other hand, if the child without a disability would be born after their siblings with disability, the existence versus non-existence logic becomes more straightforward: the child's experience with a sibling with disabilities and the only alternative to their birth is non-existence. Parents cannot be blamed for choosing to bring or not to bring the child into existence, as there is no moral subject yet to whom harm can be done.

Additionally, one might question the logic when there is already a sibling with a disability and another child with disabilities is added to the family. Adding another child to the family increases the responsibilities of parents, raising questions about their capacity in terms of time, financial resources, and support systems (Shakespeare 98). This may be especially true for children with developmental or intellectual disabilities, who may not be able to care for each other. Therefore, parents must consider whether they have the financial and emotional capacity to provide long-term care, even after their own death. While these concerns are valid, I maintain that the decision to bring a child with disabilities into the world is ethically acceptable, as long as it is made with a comprehensive understanding of the potential impacts on the entire family and based on the parents' informed decision-making.

Now it's proven that despite the suffering of the child, parents are doing no harm to the siblings of children with disabilities. However, while parents cannot be blamed for the hardships siblings face, does this mean their suffering should be taken for granted? Here, I want to address a broader question: can the ethical reasoning we apply within the family still hold when we shift to a larger societal lens, or does it require a fundamentally different framework? If the suffering of siblings is predictable and preventable at a societal level, should it be justified as it is in the context of parental decisions? To answer this question, I would like to highlight the concept of potential and actual potential that Heyd introduces to explain how the ethical impact on siblings should be considered from a broader societal perspective.

Heyd argues that when we have control over the outcome, the outcome is a potential (Heyd 100). The decision of parents to bring a child into existence is a deeply personal one, and, as Scott argues, it should ideally be free from external influence, emphasizing parental autonomy in this process (Scott 68). Therefore, a child with disabilities is a potential for parents, as their decision can influence whether or not that child is born. Heyd also clarifies that "potential people... are not just beings whose features are vague, whose character is hard to predict" (Heyd 99). The logic of unpredictability and complexity of the outcome I introduced in the earlier paragraph shows up here again to justify any decisions made by parents.

On the other hand, the child is an actual potential for the government. The government cannot control the parents' reproductive choices, but it must prepare for the fact that children with disabilities will be born and that siblings will be affected by their existence. Heyd explains that "future people are actual in the moral sense...[and] they deserve some sort of concern, as does any existing human being" (Heyd 99). This logic parallels the need for society to prepare for the future in general. We recognize the necessity of conserving energy and resources because we know humans will continue to occupy the earth in the future (Heyd 103). In the same context, society must also be ready to support the children who will inevitably be born, including those with disabilities, and their families. This logic aligns with Abbas's ethical guideline, which encourages us to ask whether we were aware of the potential consequences... and whether we could have predicted any implications of our actions (Abbas 97). While parents face personal uncertainty, leading to decisions that aim to cause no harm to their potential child with

disabilities or their siblings, governments, on a larger scale, operate with a certain level of certainty and in a position of blame if no action is taken based on plausible predictions.

Shakespeare also put emphasis on society's role by highlighting that "a poor quality of life" of people with disability could be "a consequence of social arrangements" (Shakespeare 130). The severity of a disability is rarely the sole cause of suffering (Shakespeare 130); instead, societal attitudes and support structures play a critical role. As Shakespeare claims, the government's role consists in "support, and high-quality professional services to help safely deliver the chosen outcomes" (Shakespeare 129). Parents may choose to continue a pregnancy and give birth to a child with disabilities, but if the child and their family suffer due to societal attitudes and insufficient support, then the failure lies not with the parents, but with society for not fulfilling its responsibility to provide adequate care and inclusion. Since the logic used to justify a parent's decision cannot be used to justify the failure or negative outcomes of government action or inaction, it should be able to respond to the critiques quickly to undo their wrongs.

Another concern that may arise from my claim—that both decisions, whether to terminate or continue the pregnancy, are ethical for siblings and parents should not be blamed—is whether parents are still responsible for the hardship experienced by siblings of a child with disabilities. How does the concept of blame differ from responsibility in this context? I would argue that blame occurs when harm is done, while responsibility refers to the obligation to act ethically. In this context, parents cannot be blamed because they made their best effort by making an informed decision. However, Kant's logic of good intentions requires parents to make a concerted effort to do what they perceive as the best for their child. If parents genuinely want their child to live a good life, in accordance with Kant's idea of pure intention, they must continuously make improvements and adjustments as life is long-term and involves ongoing choices. For their actions to be considered ethical beyond the prenatal decision, their choices after the birth of the child must also be ethical—essentially, they must continue to make informed decisions. To ensure their intentions are good, parents should make informed decisions and must respond to these outcomes by making follow-up informed decisions to minimize the suffering of the child, including siblings of a child with disabilities.

Addressing siblings in the ethical conversation is crucial as it acknowledges the interconnected nature of the family dynamics. When parents take siblings into account, they are respecting the life of their child and taking responsibility as a parent thoughtfully planning for their family. Not considering them will imply a neglective stance which directly contrasts the good intentions of parents making the best decision for their children. Ethical decision-making in prenatal care, then, is incomplete without acknowledging siblings' lived realities. In short, outcomes cannot be ethically justified from the perspective of siblings if their perspectives are not considered in the first place.

Throughout the paper, I proved that the ethics of prenatal decisions can be justified regardless of the outcome as long as the decision was informed. However, despite enough "informing," all lives of children born will experience a certain level of suffering, and the information gained from prenatal screening cannot be viewed as a way to eliminate that suffering. In order to

remain responsible despite the uncertainties and potential hardships, it is crucial for parents to consider multiple dimensions of family life and make an informed, thoughtful and well intended decision that accounts for the well-being of all involved. It is essential to uphold the principle that a decision should be well-informed and attentive to the broader impact on the family as a whole. While no decision can guarantee a perfect outcome, a commitment to ethical awareness—one that includes the perspectives of siblings and recognizes the interdependence of family members—can lead to decisions that are not only justifiable but deeply humane. In this light, both the decision to continue or terminate a pregnancy involving a child with disabilities can be ethically sound for siblings, as long as it is made with care, intention, and recognition of the interconnected lives involved. More broadly this claim can contribute to the larger scholarly conversation on reproductive ethics by extending the scope of ethical responsibility. To truly uphold ethical decision-making, it is essential to consider not only the immediate consequences of the decision but also the long-term impact on the family as a whole. By including siblings' perspectives and recognizing the interconnectedness of family life, we can ensure that the ethical framework surrounding prenatal decisions is more comprehensive and inclusive, offering a more humane and just approach to reproductive ethics.

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